

Dezal

Product Name

Dezal Oral Solution 0.5mg/ml

COMPOSITION

Name and Strength of Active Substance(s)

Each ml contains 0.5mg desloratadine

Product Description

Oral solution: Clear, colourless solution, free from foreign matter.

PHARMACOLOGY

Actions: Desloratadine is a non-sedating long-acting histamine antagonist with potent, selective peripheral H₁-receptor antagonist activity. It has demonstrated anti-allergic, antihistaminic and anti-inflammatory activity.

Pharmacodynamics: After oral administration, desloratadine selectively blocks peripheral histamine H₁- receptors because the drug is effectively excluded from entry to the central nervous system (CNS). In addition to antihistaminic activity, desloratadine has demonstrated anti-allergic and anti-inflammatory activity from numerous *in vitro* (mainly conducted on cells of human origin) and *in vivo* studies. These studies have shown that desloratadine inhibits the broad cascade of events that initiate and propagate allergic inflammation, including: The release of pro-inflammatory cytokines including IL-4, IL-6, IL-8 and IL-13; the release of important pro-inflammatory chemokines eg, Regulated upon Activation, Normal T-cell Expressed and Secreted (RANTES); superoxide anion production by activated polymorphonuclear neutrophils; eosinophil adhesion and chemotaxis; the expression of the adhesion molecules eg, P-selectin; IgE-dependent release of histamine, prostaglandin (PGD2), and leukotriene (LTC4); the acute allergic bronchoconstrictor response and allergic cough in animal models.

Desloratadine does not readily penetrate the CNS. At the recommended dose of 5mg daily, there was no excess incidence of somnolence as compared to placebo. Desloratadine tablet even at a dose of 7.5 mg daily, did not affect psychomotor performance. A single dose of desloratadine 5 mg did not affect standard measures of flight performance including exacerbation of subjective sleepiness or tasks related to flying.

No clinically relevant changes in desloratadine plasma concentrations were observed in multiple-dose, ketoconazole, erythromycin, fluxetine and cimetidine interaction.

Co-administration of alcohol did not increase the alcohol-induced impairment in performance or increase in sleepiness. No significant differences were found in the psychomotor test results between desloratadine and placebo groups, whether administered alone or with alcohol.

In adult and adolescent patients with seasonal allergic rhinitis (SAR), Desloratadine tablet, was effective in relieving symptoms eg, sneezing, nasal discharge and itching, congestion/stuffiness, as well as ocular itching, tearing and redness, and itching of palate. Desloratadine tablet effectively controlled symptoms for 24 hrs.

PHARMACOKINETICS

Desloratadine plasma concentrations can be detected within 30 min of desloratadine administration. It is well absorbed with maximum concentration achieved after approximately 3 hrs; the terminal phase half-life is approximately 27 hrs. The degree of accumulation of desloratadine was consistent with its half-life (approximately 27 hrs) and a once-daily dosing frequency. In adults and adolescents, the bioavailability of desloratadine was dose proportional over the range of 5-20 mg.

Desloratadine is moderately bound (83-87%) to plasma proteins. There is no evidence of clinically relevant drug accumulation following once-daily dosing of desloratadine (5-20 mg) for 14 days.

The enzyme responsible for the metabolism of desloratadine has not been identified yet, and therefore some interactions with other drugs cannot be fully excluded. *In vivo* studies with specific inhibitors of CYP3A4 and CYP2D6 have shown that these enzymes are not important in the metabolism of desloratadine. Desloratadine does not inhibit CYP3A4 or CYP2D6 and is neither a substrate nor an inhibitor of P-glycoprotein.

There is no effect of food (high-fat, high caloric breakfast) on the disposition of desloratadine. Grapefruit juice also has no effect on the disposition of desloratadine.

INDICATIONS

For the rapid relief of symptoms associated with seasonal allergic rhinitis eg, sneezing, nasal discharge and itching, congestion/stuffiness, as well as ocular itching, tearing and redness.

Dezal is also indicated for the relief of symptoms associated with chronic idiopathic urticaria eg, relief of itching and the size and number of hives.

DOSAGE AND ADMINISTRATION

Method of administration

Oral use.

The dose can be taken with or without food.

Adults and children ≥12 years: 10 mL (5 mg) once a day.

Children 6-11 years: 5 mL (2.5 mg) once a day;

Children 1-5 years: 2.5 ml (1.25 mg) once a day; The prescriber should be aware that most cases of rhinitis below 2 years of age are of infectious origin and there are no data supporting the treatment of infectious rhinitis with Dezal oral solution.

CONTRAINDICATIONS

Hypersensitivity to desloratadine or to any of the excipients of Dezal oral solution.

WARNINGS AND PRECAUTIONS

Effects on the Ability to Drive or Operate Machinery: No effects on the ability to drive and use machines have been observed.

Paediatric population

In children below 2 years of age, the diagnosis of allergic rhinitis is particularly difficult to distinguish from other forms of rhinitis. The absence of upper respiratory tract infection or structural abnormalities, as well as patient history, physical examinations, and appropriate laboratory and skin tests should be considered.

Approximately 6 % of adults and children 2- to 11-year old are phenotypic poor metabolisers of desloratadine and exhibit a higher exposure. The safety of desloratadine in children 2- to 11-years of age who are poor metabolisers is the same as in children who are normal metabolisers. The effects of desloratadine in poor metabolisers < 2 years of age have not been studied.

221395_s3 - P: Desloratadine (Dezal) MY, Oral solution, 0.5mg/ml, ALL

GENERAL INFORMATION		NON-PRINTING COLOURS	COLOURS/PLATES
Proof / Cycle:	02	1.	1.Black 
Origination Date:	10.12.2024	2.	2.
Originated by:	DY	3.	3.
Revision Date:	11,12,2024		4.
Revised by:	DY		5.
	Dimensions: 160x400 mm Supplier: Balkanpharma Troyan Min pt size*: 12 pt  DO NOT USE FOR PRINT PROOF CREATION DRAFT		6.

In the case of severe renal insufficiency, desloratadine should be used with caution.

Dezal Oral Solution contains sorbitol; thus, patients with rare hereditary problems of fructose intolerance, glucose-galactose malabsorption or sucrase-isomaltase insufficiency should not take this medicine.

USE IN PREGNANCY AND LACTATION

Pregnancy

A large amount of data on pregnant women (more than 1,000 pregnancy outcomes) indicate no malformative nor foeto/ neonatal toxicity of desloratadine. As a precautionary measure, it is preferable to avoid the use of desloratadine during pregnancy.

Breastfeeding

Desloratadine has been identified in breastfed newborns/infants of treated women. The effect of desloratadine on newborns/infants is unknown. A decision must be made whether to discontinue breastfeeding or to discontinue/abstain from desloratadine therapy taking into account the benefit of breastfeeding for the child and the benefit of therapy for the woman.

Fertility

There are no data available on male and female fertility.

DRUG INTERACTIONS

No clinically relevant interactions were observed in with desloratadine tablets in which erythromycin or ketoconazole were co-administered.

Paediatric population

Interaction studies have only been performed in adults. Desloratadine tablets taken concomitantly with alcohol did not potentiate the performance impairing effects of alcohol. However, cases of alcohol intolerance and intoxication have been reported. Therefore, caution is recommended if alcohol is taken concomitantly.

Incompatibilities: Not applicable.

ADVERSE EFFECTS

The most frequent adverse events reported were fatigue, dry mouth and headache.

Below is the tabulated list of adverse reactions:

System Organ Class	Frequency	Adverse reactions seen with desloratadine oral solution
Psychiatric disorders	Very rare	Hallucinations
Nervous system disorders	Common Common (children less than 2 years) Very rare	Headache Insomnia Dizziness, somnolence, insomnia, psychomotor hyperactivity, seizures
Cardiac disorders	Very rare Not known	Tachycardia, palpitations QT prolongation
Gastrointestinal disorders	Common Common (children less than 2 years) Very rare	Dry mouth Diarrhoea Abdominal pain, nausea, vomiting, dyspepsia, diarrhoea
Hepatobiliary disorders	Very rare Not known	Elevations of liver enzymes, increased bilirubin, hepatitis Jaundice
Skin and subcutaneous skin disorders	Not known	Photosensitivity
Musculoskeletal and connective tissue disorders	Very rare	Myalgia
General disorders and administration site conditions	Common Common (children less than 2 years) Very rare Not known	Fatigue Fever Hypersensitivity reactions (such as anaphylaxis, angioedema, dyspnoea, pruritus, rash, and urticaria) Asthenia

OVERDOSE AND TREATMENT

In the event of overdose, consider standard measures to remove unabsorbed active substance. Symptomatic and supportive treatment is recommended.

In adults and adolescents, up to 45 mg of desloratadine was administered (9 times the clinical dose) and no clinically relevant effects were observed.

Desloratadine is not eliminated by hemodialysis; it is not known if it is eliminated by peritoneal dialysis.

STORAGE:

Store in the original package below 30°C

AVAILABILITY

0.5mg/ml oral solution: Bottle of 60ml and 100ml

For further information, please consult your physician or pharmacist.

MANUFACTURED BY:

Balkanpharma Troyan AD
1 Krayrechna Str.
Troyan 5600
BULGARIA

DATE OF REVISION: December 2024

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